

Discount Approval Policy

Document ID: FIN-POL-004 Version: 1.0 Effective Date: 01 June 2026 Department: Finance & Billing
Classification: Internal Document Owner: Chief Financial Officer

1. Purpose

This policy defines the controls used by MedCore Hospitals to request, evaluate, approve, record and monitor discretionary discounts applied to patient bills. It establishes who may authorize a discount, what evidence must support a request, how different types of discounts are treated, and how exceptions are escalated. The objective is to provide a consistent approval trail without preventing legitimate account adjustments from being handled promptly.

The policy applies specifically to discretionary concessions. Contractual rates already agreed with an insurer, corporate account, or empanelled scheme are handled under their approved tariff or package terms and are not treated as case-by-case discounts.

2. Scope and Applicability

The policy applies to billing executives, billing supervisors, Finance Managers, insurance desk personnel, hospital administrators and other employees who initiate, recommend, approve or record discounts. It covers inpatient, outpatient, diagnostic, day-care and emergency accounts where a discretionary reduction is proposed.

- A discount may be considered only against an identified patient account and a specific charge or bill.
- The policy governs financial approval; it does not authorize clinical changes, alteration of treatment records or deletion of valid service entries.
- Discounts must be recorded in the Hospital Management System (HMS) before or at the time they are applied, according to the applicable workflow.
- A discount cannot be used to conceal an incorrect charge. Incorrect charges should first be corrected through the relevant billing process.

3. Key Definitions

Term	Meaning within this policy
Discretionary discount	A reduction granted through an internal approval decision rather than through a pre-agreed tariff or package.
Contractual rate	A rate already agreed with a corporate, insurer, TPA or empanelled scheme and therefore not treated as a discretionary concession.
Discount request	A documented proposal identifying the account, amount, reason and requested approving authority.
Discount band	The percentage range or approval threshold used to determine the minimum approving authority.
Manual override	A change entered outside the standard automated rate or discount configuration and therefore requiring additional control.
Waiver	An authorized decision to remove a specified financial obligation; where used, it follows the same documentation discipline as a discount.

4. Discount Classification

Before approval, the requester identifies why the proposed reduction is being made. Classification prevents staff from applying a discretionary discount where the amount should instead be corrected, settled under a contract, or handled through another policy.

Category	Examples	Normal treatment
Billing correction	Duplicate charge, wrong service code, charge not supported by service record	Correct the underlying bill; do not disguise the correction as a discount.

Contractual concession	Corporate tariff, insurer package, empanelled scheme rate	Apply approved contractual terms; discretionary matrix does not apply.
Service recovery	Documented service issue resulting in an approved financial concession	Requires reason, account reference and approval under this policy.
Financial hardship request	Patient requests reduction based on an approved internal hardship assessment	Consider only where supporting evidence and authority requirements are met.
Exceptional commercial adjustment	Unusual account circumstance requiring management approval	Treat as an exception where it falls outside routine bands.

5. Standard Approval Matrix

MedCore's standard discount authority is determined by the percentage reduction proposed against the eligible bill or charge. The percentage is calculated against the amount to which the discount applies, not against unrelated balances.

Discount level	Minimum approval authority	Required record
Up to 5%	Billing Supervisor	Reason, affected charge/bill and HMS approval entry
Above 5% and up to 10%	Finance Manager	Business/patient rationale, calculation and HMS approval entry
Above 10% and up to 20%	Hospital Administrator	Detailed justification, prior review and HMS approval entry
Above 20%	Chief Financial Officer	Detailed justification, reconciliation and documented exception rationale where applicable

The authority bands above are aligned with the fictional Hospital Billing Policy. That policy states that discounts up to 5% require Billing Supervisor approval, discounts above 5% and up to 10% require Finance Manager approval, discounts above 10% and up to 20% require Hospital Administrator approval, and discounts above 20% require Chief Financial Officer approval.

6. How the Discount Percentage Is Determined

The requester must state the base amount and the proposed reduction. For a simple example, a 10% discount on an eligible charge of ₹40,000 produces a proposed reduction of ₹4,000 and a post-discount amount of ₹36,000. If the bill contains several components, the request should identify which components are included rather than applying a percentage to an unrelated total.

Where an existing contractual tariff or package rate has already reduced the price, the discretionary discount percentage is calculated only after the contractual treatment has been correctly reflected. The requester must not stack an unapproved discretionary concession onto a contractual adjustment merely because the combined reduction appears within a lower approval band.

7. Initiating a Discount Request

A discount request begins with the employee handling the patient account. The requester records the patient/account identifier, bill or transaction reference, gross amount relevant to the request, proposed discount amount, percentage, reason, supporting information and the name of the proposed approver.

- The account must be identifiable in the HMS.
- The requester must distinguish a genuine discount from an incorrect charge that needs correction.
- The proposed amount and percentage must be mathematically consistent.
- Any linked insurance, corporate or scheme arrangement must be checked before a discretionary discount is proposed.
- The request must not be split into multiple smaller requests merely to remain within a lower approval band.

8. Supporting Rationale

The rationale should explain why a discount is being requested and why the requested level is appropriate. Statements such as "patient requested discount" or "management instruction" are not sufficient on their own for higher-value requests. The record should identify the relevant circumstances and the financial impact.

Situation	Useful evidence or explanation
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Service recovery	Reference to the documented service issue and the authorized resolution.
Financial hardship	Internal hardship assessment or other approved evidence available to the decision-maker.
Billing concern	Description of the concern and confirmation that the issue is not simply an incorrect charge.
Exceptional account circumstance	Specific facts, financial impact and why normal treatment is insufficient.
Management-directed concession	Named authorized decision-maker and documented reason for the direction.

9. Review by the Approver

The approver reviews the request before the discount is applied. The review should establish that the request is within the approver's authority, the calculation is correct, the underlying charge is valid, and the stated reason is sufficiently documented.

- Confirm the bill/account reference and eligible base amount.
- Recalculate the proposed percentage and discount value.
- Check whether a contractual rate, insurance settlement or other adjustment already applies.
- Check for duplicate or related discount requests on the same account.
- Assess whether the request should instead be handled as a billing correction, refund, write-off or another approved transaction.
- Record the decision and reason in the HMS.

10. Segregation of Duties

The employee who prepares or enters a discount request should not approve the same request when a separate authorized approver is available. The Hospital Billing Policy establishes segregation of duties as a billing principle and requires adjustments, discounts and write-offs to be logged with the approving authority and a stated reason.

Where operational constraints make complete separation impractical, the case must be handled under the applicable exception process and subject to retrospective review by a higher Finance authority. Such a review does not convert an unauthorized discount into an approved one.

11. Emergency and Time-Sensitive Accounts

A genuine emergency must not be delayed because a discount has not yet been approved. Clinical and emergency operating procedures take precedence over financial approval timing for immediate care. If a discount is considered after stabilization, it follows the normal financial approval path.

For a discharge that is otherwise ready but has an unresolved discount request, billing staff should not assume that the requested discount is approved. The account remains subject to the approved amount until an authorized decision is recorded.

12. Discounts on Insured, Corporate and Scheme Accounts

Discounts involving third-party payment arrangements require additional review because the amount ultimately payable by the patient may depend on authorization, package terms or contractual tariff. The Insurance Claims SOP describes situations in which approved cashless amounts, co-payments and non-payable items determine the patient's financial responsibility.

A contractual insurer, TPA, corporate or empanelled-scheme rate is not a discretionary discount. Staff must not label a contractual rate as a discount simply to route it through the discretionary matrix.

Conversely, where a genuine discretionary concession is requested on a patient-responsibility amount, the request must identify the amount to which the concession applies and the relevant third-party settlement status.

13. Interaction with Refunds

A discount changes the amount payable; a refund returns money that has already been received in excess of the reconciled amount. If a discount is approved after the patient has already paid the affected amount, the revised account may create a refundable credit. The discount approval and the subsequent refund are separate transactions and should retain their own references.

The Patient Refund Policy governs recipient verification, refund approval, payment processing and refund closure. Finance should therefore avoid using a refund entry to represent a discount or using a discount entry to represent a refund.

14. Prohibited Practices

- Splitting one discount into multiple requests to avoid the correct approval level.
- Backdating an approval without a documented reason and authorized review.
- Changing a service charge merely to create the appearance of an approved discount.
- Applying a discount before the required approval has been recorded.
- Using another employee's approval credentials.
- Approving a discount on a charge that is already under unresolved dispute without addressing the dispute workflow.
- Creating a discretionary discount for a contractual tariff that should be handled under the relevant agreement.
- Deleting or overwriting the original discount record when a correction is required.

15. Exception Management

Any deviation from the standard discount matrix or process must be documented. The fictional Hospital Billing Policy requires written Chief Financial Officer approval for deviations such as a discount, waiver or billing timeline outside the stated parameters, with the deviation recorded in the Finance exceptions log.

Exception type	Required action	Escalation
Discount above normal authority	Hold application until higher authority approves.	Chief Financial Officer
Missing supporting information	Return request for completion.	Approver / Finance Manager
Potential duplicate discount	Place request on hold and reconcile related entries.	Finance Manager
Conflicting third-party settlement	Confirm insurance/corporate position before final decision.	Finance Manager + Insurance Desk
Manual override after approval	Record reason and retain original approval trail.	Higher Finance authority
Suspected misuse of approval	Freeze affected request and preserve records.	Finance Manager / CFO

16. Monitoring and Periodic Review

Finance reviews discount activity periodically to identify unusual patterns rather than evaluating each discount only in isolation. Monitoring may include discount volume by employee, department, patient account, percentage band, reason category and time period.

- Repeated requests just below an approval threshold should be reviewed for possible threshold avoidance.
- Unusually high discounts in one department should be compared with normal account activity and documented reasons.
- Cancelled and re-entered discounts should be included in review because they may indicate process weaknesses.
- Discounts followed quickly by refunds should be sampled to confirm that both transactions are correctly linked.
- Changes to tariffs, packages or approval limits should be reflected in the applicable master data and communicated to billing staff.

17. Audit Trail and Record Retention

Every approved discount should leave an auditable trail showing who requested it, what account it affected, what amount was proposed, why it was requested, who approved it, when it was approved and what amount was ultimately posted. Supporting documents should remain linked or traceable to the transaction.

The fictional Hospital Billing Policy states that billing records, including discount approvals, are retained for a minimum of eight years from the date of the final invoice, subject to the relevant MedCore records and access policies. This policy follows that record-retention direction for discount records.

18. Frequently Encountered Scenarios

Scenario	Guidance
A patient asks for a 4% reduction on a valid bill.	A Billing Supervisor may approve the discretionary discount if the request is documented and no other policy dependency prevents it.
A patient asks for a 15% reduction.	The request falls within the above-10%-to-20% band and therefore requires Hospital Administrator approval.
A corporate agreement already provides a reduced tariff.	Treat the agreed tariff as contractual rather than as a discretionary discount.
A billing executive wants to correct a duplicate charge.	Correct the underlying billing error rather than recording the correction as a discount.
A 12% discount was entered without approval.	Do not treat the entry as valid approval; escalate and document the exception/correction.
A discount is approved after the patient has already paid.	Post the approved adjustment correctly. If it creates a verified credit balance, handle the resulting refund under the Patient Refund Policy.
Several 5% requests are submitted against related charges on one account.	Review the combined request rather than assuming each entry independently qualifies for the lowest approval band.

19. Approval Checklist

Before approving a discretionary discount, the authorized reviewer should confirm the following:

- The account and affected charge are clearly identified.
- The underlying charge is valid and is not simply an error requiring correction.
- The discount category and reason are documented.
- The percentage and amount have been recalculated.
- Any insurer, TPA, corporate or scheme dependency has been considered.
- The proposed discount is within the approver's authority.
- The request is not part of a split or duplicate approval pattern.
- The approval is recorded using the authorized workflow and credentials.
- Any exception is separately documented and escalated.
- If the discount creates a credit after payment, the refund is processed separately under the PatientRefund Policy.

20. Related Documents

- Hospital Billing Policy (FIN-POL-001)
- Insurance Claims SOP (FIN-SOP-002)
- Patient Refund Policy (FIN-POL-003)
- Employee Reimbursement Policy (FIN-POL-005)
- Patient Complaint Resolution Policy (PS-POL-003)
- Regulatory Compliance Manual (LEG-MAN-005)

21. Document Control and Revision History

Version	Date	Summary of change	Owner
1.0	01 June 2026	Initial release; established discretionary discount categories, authority bands, controls, exception handling and monitoring requirements.	Chief Financial Officer